
Nervous System Self-Assessment

Learn to read your own body's stress signals —
and what they actually mean

NATE RATCLIFF, LMT

Nervous System Self-Assessment

Learn to read your own body's stress signals — and what they actually mean

NATE RATCLIFF, LMT

HEALING HANDS BY NATE

Nervous System Self-Assessment

© 2026 Healing Hands By Nate. All rights reserved.

Nate Ratcliff, LMT

Inner Circle Deep Cut — #4

For Healing Hands By Nate Inner Circle members. Personal use only. Please don't redistribute — share the membership instead.

Published by Healing Hands By Nate

Contents

1. Your Nervous System in 5 Minutes (The Basics, No Jargon)
2. The Three States: Fight-Flight, Freeze, and Rest-Digest
3. How to Read Your Own Signals
4. The Weekly Self-Assessment Protocol
5. What Each Pattern Means and What to Do
6. Building Your Personal Baseline: What's Normal For You
7. Seasonal Nervous System Patterns
8. When Self-Help Isn't Enough — Mental Health Referral
9. Stories From the Table
10. What to Expect in a Year of Consistent Work
11. Closing

Introduction

AN INNER CIRCLE DEEP CUT. A SELF-GUIDED LITERACY resource for understanding your own nervous system patterns.

After 12+ years on the table, I've come to believe that nervous-system literacy is one of the most under-rated forms of self-knowledge a person can develop. People know how their car runs. They know the names of muscles they don't even use. They have opinions about politics and sports and parenting. They don't know what state their nervous system is in right now — or what to do about it.

That's not a moral failing. Nobody taught us. Nervous-system regulation wasn't on the school curriculum. Most adults grew up in households where "feelings" were what you had and "your body" was something separate that occasionally got sick. The idea that your nervous system has discrete states, that those states drive your physical experience, and that you can deliberately influence which state you're in — that's relatively new knowledge in mainstream culture.

This deep cut is to give you that knowledge in usable form. Not academic theory. Practical self-assessment. By the end of this document, you'll be able to read your own nervous system the way you read the weather — and respond appropriately to what you see.

One important note up front: This is educational, not therapeutic. I'm a licensed massage therapist, not a mental health practitioner. If what you find through this self-assessment points to clinical anxiety, depression, PTSD, or other mental health concerns, those need a qualified mental health professional. This document teaches you the framework. Treatment of clinical issues is outside my scope — and outside what this document is trying to provide.

CHAPTER 1

Your Nervous System in 5 Minutes (The Basics, No Jargon)

THE NERVOUS SYSTEM IS YOUR BODY'S COMMUNICATION network. It connects your brain to everything else through nerves. There are several ways to slice it, but for our purposes, two divisions matter:

Voluntary vs. autonomic

The voluntary nervous system controls things you consciously do — moving your arm, walking, talking. You think about it, it happens.

The autonomic nervous system controls things you don't consciously do — heart rate, breathing, digestion, blood pressure, immune function. It runs automatically in the background.

The autonomic system is where nervous-system regulation lives. You don't consciously decide your heart rate, but your nervous system does — based on signals about whether you're safe or under threat.

Three branches of the autonomic system

For decades, scientists divided the autonomic system into two states:

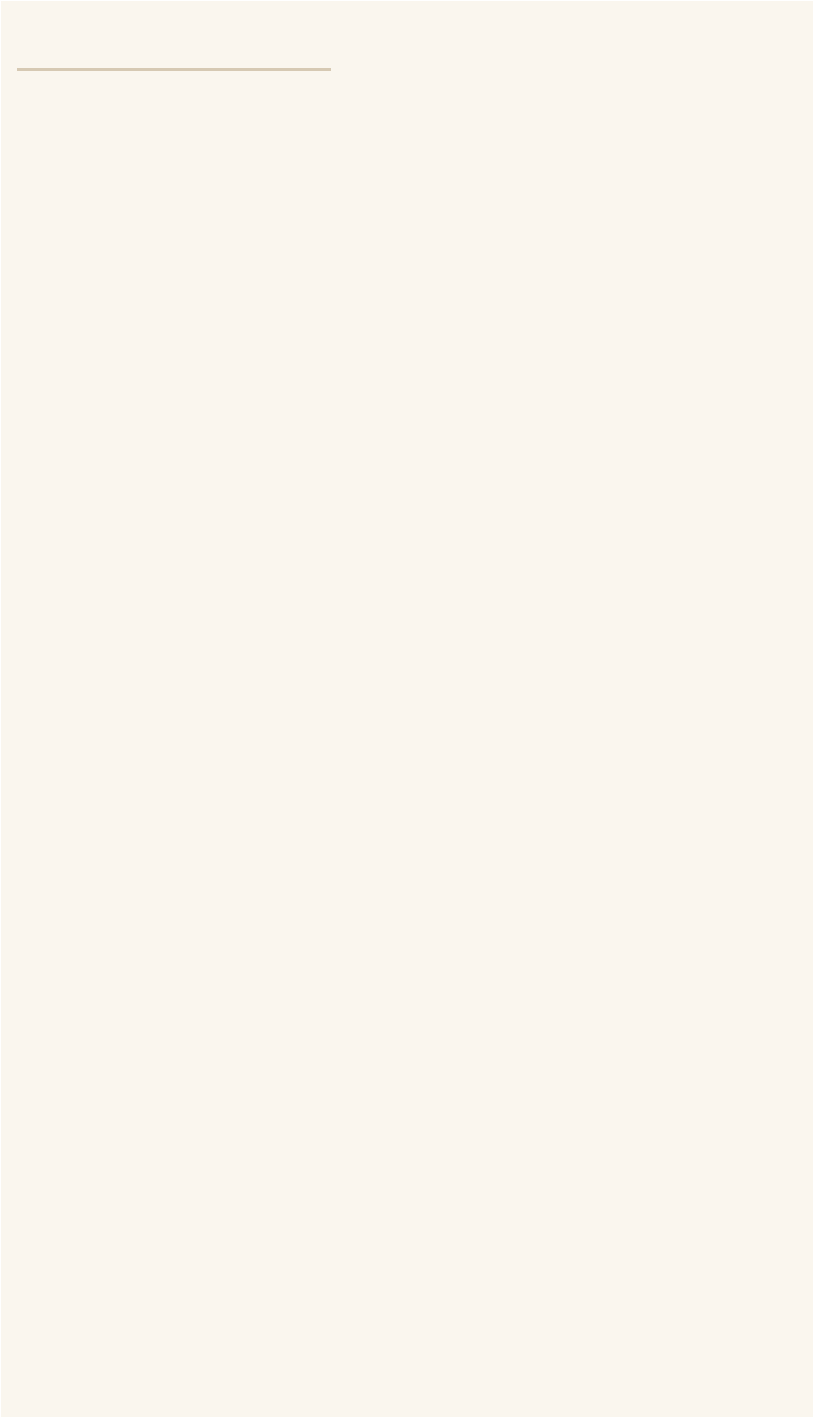
- **Sympathetic** (“fight or flight”) — activation, energy mobilization, alertness
- **Parasympathetic** (“rest and digest”) — recovery, digestion, sleep, repair

In the 1990s, neuroscientist Dr. Stephen Porges proposed what’s called Polyvagal Theory, which expanded the model into three states:

- **Ventral vagal** (“safe and social”) — calm, connected, engaged, present
- **Sympathetic** (“mobilized”) — fight or flight activation
- **Dorsal vagal** (“shutdown”) — freeze, collapse, immobilization

This three-state model is now widely used in trauma therapy, stress research, and somatic practice because it better matches what people actually experience. Most people have all three states in their behavioral repertoire — but most have a “default” they fall into when stressed.

This three-state model is what we’ll use throughout this deep cut. It’s clinically useful and matches lived experience more accurately than the older two-state model.



CHAPTER 2

The Three States: Fight-Flight, Freeze, and Rest-Digest

LET'S GO DEEPER INTO EACH STATE — WHAT IT looks like, feels like, and means.

State 1 — Ventral Vagal (Safe and Social)

This is your baseline when everything is okay. Your nervous system has assessed the environment and decided “we’re safe, we can engage.”

Physical signals: - Steady, easy breathing — belly moves naturally - Heart rate at relaxed baseline - Warm hands and feet (good peripheral circulation) - Easy eye contact - Soft facial expression - Voice has natural prosody (rises and falls) - Digestion working normally

Mental/emotional signals: - Present, engaged - Curious about the world - Able to feel both your own emotions and respond to others’ - Sense of “I’m here, you’re here, we’re okay” - Genuine interest, genuine connection

What it feels like: “I feel like myself.” That sense of being at home in your own body. Available for what’s happening.

This is the state your body needs to spend time in for recovery, repair, connection, and creative engagement. Most adults in modern life don’t spend enough time here.

State 2 — Sympathetic (Fight or Flight)

This is the activation state. Your nervous system has detected a threat (real or perceived) and is mobilizing energy to respond.

Physical signals: - Faster, shallower breathing - Elevated heart rate - Cold hands and feet (blood routed to core) - Muscle tension, especially in jaw, shoulders, hands - Pupils dilated - Difficulty sitting still - Reduced digestion - Heightened sensitivity to sound/light

Mental/emotional signals: - Anxious, irritated, angry - Mind racing, planning, problem-solving - Hypervigilant — scanning for what’s wrong - Reactive — small things feel large - Energized but uncomfortable

What it feels like: “Wired.” “On edge.” “Can’t slow down.” That hyperactive feeling that often comes with stressful periods.

The sympathetic state isn’t bad — you NEED it. Acute stress mobilizes you to handle real challenges. The problem is when you live there. Chronic sympath-

etic dominance is what most modern adults experience as “stress.”

State 3 — Dorsal Vagal (Freeze / Shutdown)

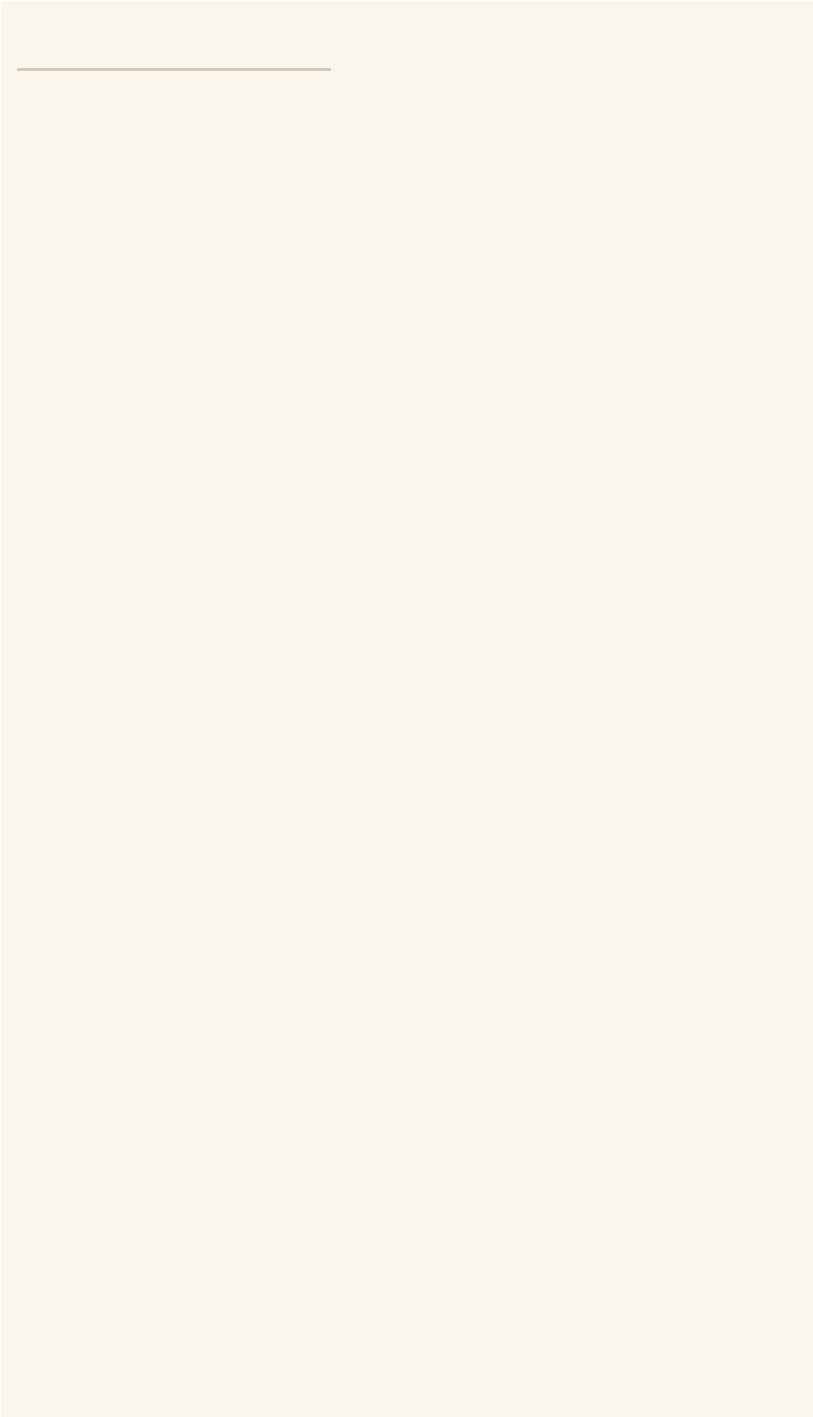
This is the immobilization state. Your nervous system has assessed that fight or flight wasn’t going to work — the threat was too overwhelming — and shifted into shutdown mode.

Physical signals: - Very slow breathing (sometimes you forget to breathe) - Slowed heart rate - Cold or numb extremities - Feeling of heaviness, sluggishness - Foggy thinking - Fatigue not relieved by sleep - Digestive shutdown or dysfunction - Disconnection from physical sensation

Mental/emotional signals: - Flat, numb, “checked out” - Depressed, hopeless, helpless - Unable to access emotions clearly - Disengaged from people and surroundings - Sense of “going through the motions”

What it feels like: “Numb.” “Heavy.” “Going through the motions.” That state where everything feels like effort and nothing feels alive.

This state is biologically protective — when a threat is overwhelming, shutdown is the body’s last-resort defense (think of an animal “playing dead”). It’s useful in actual emergencies. It’s NOT useful as a chronic state, which it often becomes after sustained stress or trauma.



CHAPTER 3

How to Read Your Own Signals

THE FIRST STEP IN NERVOUS-SYSTEM LITERACY IS just observing yourself. Without judgment. Without trying to change anything yet.

The five-question check-in

Ask yourself, right now:

1. **What's my breathing doing?** Fast or slow? Deep or shallow? Belly moving or just chest?
2. **What's my heart rate doing?** Can I feel it? Is it fast or slow? Steady or racing?
3. **What are my hands feeling?** Warm or cold? Loose or tight?
4. **What's my mental state?** Engaged or scattered? Calm or wired? Numb or activated?
5. **What's my facial expression?** Soft or tense? Mouth relaxed or jaw clenched? Eyes soft or hard?

Run this check three times a day for a week and you'll start seeing patterns. You'll know whether you defaulted to sympathetic activation or dorsal shutdown during stressful events. You'll catch yourself in states you didn't know you were in.

Look for combinations

Most people aren't in pure states — they have combinations.

Common combination 1 — Sympathetic + dorsal stacked: Anxious AND exhausted. Mind racing AND body shut down. “Wired but tired.” This is what chronic stress often looks like over time.

Common combination 2 — Brief ventral interrupted: Feeling okay for short stretches but easily triggered back into sympathetic by small things. The threshold for activation is low.

Common combination 3 — Mostly ventral with situational sympathetic: Generally regulated but specific triggers (certain people, certain situations) reliably activate. This is closer to healthy regulation.

Knowing your combination matters more than identifying a single state, because the interventions differ.

CHAPTER 4

The Weekly Self-Assessment Protocol

THIS IS YOUR TOOL FOR ONGOING NERVOUS-SYSTEM literacy. Do this once a week — Sunday evening works well — for as long as you want to track your patterns.

Takes 10 minutes. Gives you data over time.

Step 1 — Quick state assessment right now

Use the five-question check-in above. What state am I in right now?

- Mostly ventral vagal (safe/social)
- Sympathetic (activated)
- Dorsal vagal (shut down)
- Combination (specify)

Step 2 — This week's predominant pattern

Look back at the past week. What was the overall pattern?

- Mostly regulated (ventral majority of the time)

- Sympathetic-dominant (activated most of the week)
- Dorsal-dominant (shut down most of the week)
- Volatile (swinging between states)
- Wired-and-tired (sympathetic + dorsal stacked)

Step 3 — Triggers I noticed this week

What predictably activated me OR shut me down this week?

People: _____ **Situations:** _____ Events: _____
Body states: _____

Step 4 — What worked this week to shift my state

What did I do that helped me regulate? (Even if I didn't notice it at the time.)

Step 5 — What didn't work that I want to try differently

What patterns did I fall into that I want to interrupt next week?

CHAPTER 5

What Each Pattern Means and What to Do

If you're sympathetic-dominant most of the time

WHAT IT MEANS: YOUR NERVOUS SYSTEM IS stuck in mobilization mode. Chronic activation. Your body is constantly preparing for threats that may or may not be present. Over time this leads to: chronic muscle tension, sleep disruption, digestive issues, mood volatility, immune dysfunction, accelerated wear on the cardiovascular system.

What helps: - Vagus nerve activation work (Module 4) - Diaphragmatic breathing (Module 2) - Bodywork that focuses on parasympathetic activation - Reducing or eliminating chronic stressors where possible - Adequate sleep (Module 3) - Vagal tone training over weeks/months

If you're dorsal-dominant most of the time

What it means: Your nervous system has decided that fight/flight isn't working, so it's shifted to shut-down. This often happens after sustained overwhelming stress, trauma, or chronic feelings of helplessness. Over time this leads to: depression-like symptoms, fatigue, social withdrawal, disconnection from self and others, physical numbness.

What helps: - Gentle activation work — short walks, low-intensity movement - Connection with safe people (social engagement activates ventral vagal) - Light exposure, especially morning sun - Touch — including bodywork, but also hugs, hand-holding with safe people - Sometimes counterintuitively: very light sympathetic activation (cold water, brief intense exercise) to “wake up” the system before parasympathetic recovery - **If symptoms are significant, this often warrants professional mental health support.** Dorsal-dominance can correlate with depression and trauma responses that benefit from therapy.

If you're swinging between states (volatile)

What it means: Your nervous system has lost its ability to maintain a steady ventral baseline. Small triggers send you into sympathetic or dorsal quickly, and you spend more time transitioning than resting.

What helps: - Consistent daily regulation practices (breathing, movement, vagal work) - Predictable routine to reduce nervous-system load - Bodywork to reduce baseline tension - Identifying triggers and reducing exposure where possible - Often benefits from professional mental health support to address underlying patterns

If you're wired-and-tired (sympathetic + dorsal stacked)

What it means: Both systems are running at once — activation without ability to discharge. Often the result of pushing through sustained stress for too long. Common in caregivers, parents of young children, people in high-stress jobs.

What helps: - Genuine rest (not “rest” with the phone in hand) - Vagal activation - Gentle movement (not intense exercise — that adds more sympathetic load) - Reducing demands where possible - Addressing sleep - Sometimes warrants medical evaluation for adrenal function, thyroid, or other contributing factors

If you're mostly ventral with situational sympathetic

What it means: You're well-regulated overall but have specific triggers that reliably activate you. This is closer to healthy nervous-system function than the patterns above.

What helps: - Continue what's working - Address specific triggers when possible - Use acute interventions (Module 4's vagal techniques) in moments of activation - Maintain the practices that keep you in ventral baseline

CHAPTER 6

Building Your Personal Baseline: What's Normal For You

HERE'S THE PART MOST PEOPLE MISS: THERE'S no universal "right" nervous system state. Your healthy baseline is yours.

Some people have naturally higher baseline activation (faster heart rate, more energized as default). Some have naturally calmer baselines (slower heart rate, more contemplative as default). Neither is wrong.

What matters is **your** baseline — and noticing when you're significantly off it.

Establishing your baseline

For two weeks, do the five-question check-in three times a day. Note your typical state at each time.

Morning baseline: What's your default state when you wake up? **Mid-day baseline:** What's your default

in the early afternoon? **Evening baseline:** What's your default before bed?

After two weeks, you'll know your typical pattern. That's your reference point.

Noticing significant drift

When your state shifts significantly from your baseline: - For more than 3-4 days continuously - Without an obvious trigger - In a direction that doesn't resolve with normal interventions

That's worth paying attention to. It can indicate: - Cumulative stress catching up with you - Burnout setting in - A new health issue (sleep dysfunction, hormonal change, illness) - Something in your life that needs addressing - Mental health concern that warrants professional support

The baseline gives you a reference. The drift from baseline is the data.

CHAPTER 7

Seasonal Nervous System Patterns

YES, THE NERVOUS SYSTEM HAS SEASONAL PATTERNS too. Knowing them helps you prepare.

Spring (March-May)

Common pattern: Sympathetic activation increases. More daylight, more activity, more demands. People who've been dorsal through winter often shift into sympathetic — which feels good initially (“I have energy again!”) but can tip into anxiety if not regulated.

Strategy: Pace the activation. Add structure. Don't take on too much too fast.

Summer (June-August)

Common pattern: Variable. Some people thrive in summer (extended daylight = more ventral vagal activation). Others get more sympathetic from the constant social/activity demands.

Strategy: Notice your personal pattern. Some people need to schedule rest more deliberately in summer. Others naturally find it.

Fall (September-November)

Common pattern: Often a transition period. Daylight decreases. Activity demands often decrease. Some people get more ventral (welcome quiet). Others slide toward dorsal as the world darkens.

Strategy: This is when seasonal affective patterns can start showing up. Light exposure becomes more important. Maintain physical activity even as the weather discourages it.

Winter (December-February)

Common pattern: Dorsal tendencies more common. Less light, less movement, more isolation. Holidays add sympathetic load on top of the underlying tendency to shut down. Common to have wired-and-tired stacked states.

Strategy: Light exposure (morning sun or light box), movement despite weather, social connection, the year-end nervous system reset (Module 12).

CHAPTER 8

When Self-Help Isn't Enough — Mental Health Referral

THIS IS CRITICAL, AND WORTH SAYING CLEARLY.

Nervous system literacy and self-regulation practices are powerful. They work for a wide range of stress and dysregulation. But they are not therapy. They are not treatment for clinical mental health conditions.

Seek mental health support if:

- Persistent depression or hopelessness lasting more than 2-3 weeks
- Anxiety that significantly impairs daily function
- Trauma symptoms (intrusive memories, hypervigilance, emotional flashbacks, avoidance) that are interfering with life
- Suicidal thoughts — please contact 988 (Suicide and Crisis Lifeline) or your nearest ER
- Patterns of dorsal shutdown that don't respond to self-help and are impacting your work, relationships, or basic functioning

- History of trauma that you've been managing but is starting to surface
- Substance use that's increasing in response to stress

Where to find support:

- Your primary care physician can refer to mental health professionals and assess whether medication might help
- Therapy directories: Psychology Today, Therapy Den, BetterHelp (telehealth)
- Trauma-specific approaches: EMDR therapists, somatic experiencing practitioners
- Many therapists now do telehealth, expanding access in rural areas

What I can do:

I can provide bodywork that supports nervous-system regulation. I can teach you the principles in this document. I can be one part of a recovery team.

What I can't do:

Treat clinical mental health conditions. That's outside my scope as a licensed massage therapist. If your self-assessment points to a clinical issue, please get qualified professional support.

The two work well together: bodywork addresses the body side of nervous-system dysregulation; therapy addresses the cognitive/emotional/psychological

side. Many of my clients work with both. They benefit from the combination.

CHAPTER 9

Stories From the Table

“I didn’t know I was dorsal-shut-down — I thought I was ‘fine’”

A CLIENT CAME IN FOR CHRONIC SHOULDER TENSION. We worked the shoulders. Over a few sessions, they started noticing things they hadn’t before — that they were going through the motions of life, that they weren’t really feeling things, that they couldn’t remember the last time they laughed genuinely.

That wasn’t fine. That was dorsal vagal dominance, probably for years.

Bodywork helped them notice. It didn’t fix the underlying pattern. They started therapy. The combination of bodywork + therapy over the following year shifted them significantly. They felt things again.

The lesson: sometimes you don’t know what state you’re in until you start having something to compare it to. Awareness comes first.

“I thought all the tension WAS me — I didn’t know it was a state”

Another client lived in chronic sympathetic activation. They were “high-energy” — that’s how they described themselves. They were also chronically tense, sleeping poorly, and had a string of stress-related health issues.

They thought “high-energy” was their personality. It turned out to be a nervous-system state that had become baseline.

After a year of practice (bodywork, breathwork, addressing some underlying stressors), they identified as “lower-energy than I used to be.” But more accurate version: they were now able to access ventral baseline. The “high-energy” was actually sympathetic dominance they’d mistaken for personality.

The lesson: chronic states can feel like identity. They’re not. They’re patterns that can be shifted.

“Reading my body literally changed my marriage”

A client started doing the daily check-ins. Within a month, they noticed they consistently went sympathetic during certain conversations with their spouse. Not because the spouse was doing anything wrong — but because of an old activation pattern.

Once they could see it, they could pause. Take a few slow exhales. Re-regulate. Then re-engage the conversation from ventral instead of sympathetic.

Their marriage improved noticeably within a few months. Same partner. Same conversations. Different nervous-system state.

The lesson: nervous-system awareness changes relationships, because it changes how you show up. You can't change other people. You can change which state YOU show up in.

CHAPTER 10

What to Expect in a Year of Consistent Work

MONTH 1-2: BUILD AWARENESS. IDENTIFY YOUR default state and your typical patterns. Notice when you shift.

Month 3-4: Start using interventions in real-time. Catch sympathetic activation early and use breathing/vagal techniques to shift. Notice dorsal patterns and gently activate.

Month 5-6: Baseline starts shifting. You spend more time in ventral than you used to. Less time in dysregulated states.

Month 7-12: The framework becomes internal. You don't need to think about it as much — the regulation becomes automatic. You catch states quickly and shift without effort. Your relationships, work, and physical health all benefit from the underlying stability.

This is realistic, not optimistic. The trajectory of a nervous system that gets attention is dramatically different from one that doesn't.

CHAPTER 11

Closing

YOUR NERVOUS SYSTEM IS THE OPERATING SYSTEM underneath everything else in your body. Your muscles, your breathing, your digestion, your mood, your sleep, your immune function, your relationships — all influenced by what state your nervous system is in.

Learning to read it is one of the most leveraged forms of self-knowledge you can develop. The practices that influence it are mostly free and available anytime. The benefits compound over months and years.

This document is a starting framework. The deeper work — for some people, especially those with trauma history or chronic mental health concerns — requires qualified professionals. Don't try to do that alone if it's what you need.

For the rest, the daily and weekly practices in this document, paired with the bodywork I provide and the techniques from the monthly modules, are enough to dramatically shift your nervous-system patterns over time.

The companion workbook has a structured weekly self-assessment template, a pattern tracker, and three reflection prompts to help you understand your own nervous-system signature.

— Nate



This book was crafted by Nate Ratcliff,
LMT and published by Healing Hands By
Nate.